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2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

MIDWIFERY



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MINISTRY OF HEALTH - ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization

Purpose of the Information Booklet

The EHPLE Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPLE process.

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Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

-
- Candidates are allowed



- Attend the orientation session in order to sit for the exam
 - Arrive at the exam center on time
 - Bring a legal ID and admission paper
 - Complete the exam within the allotted time frame
-

➤ Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3. Exam Framework

The key broader professional roles, also known as domains or main knowledge areas serve as a building framework for the licensing examination content for *Midwifery professionals*. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains) defining the professional identity of *Midwives*. Tasks specifying the performance level of each sub-domain serve as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis/weight for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key professional roles/ domains

- ☐ Patient Care (70.5%)
- ☐ Scholar (6.0%)
- ☐ Professionalism (5.5%)
- ☐ Leadership and management (5.0%)
- ☐ Health promotion and disease prevention (13.0%)

Key role/ domain 1: Patient care (70.5%)

Description: This domain encompasses the professional roles of midwives in the provision of high-quality, safe, and patient-centered maternal, newborn, and child health care within their scope of practice. The provision of up-to-date, ethical, and resource-efficient midwifery care requires the application of integrated knowledge of biomedical, clinical, behavioral, and social sciences. This is conducted in collaboration with patients and their families, other healthcare professionals, and the community. As patient care providers, midwives shall implement the midwifery care process by collecting and interpreting information, making clinical decisions, carrying out diagnostic and therapeutic interventions, providing midwifery care, and evaluating interventions. To demonstrate competence in this domain, the candidate shall apply such integrated knowledge in the following sub-units/domains:

- ☐ Preconception care (5.0%)
- ☐ Family planning (8.5%)
- ☐ Antenatal care (12.0%)
- ☐ Intrapartum care (15.5%)
- ☐ Post-partum care (8.0%)
- ☐ Neonatal & Childcare (9.0%)
- ☐ Gynecologic & abortion care (8.5%)
- ☐ Fundamental Midwifery skills (4.0%)

Key role/ domain 2: Scholar (6.0%)

Description: This domain encompasses the professional role of *Midwifery professionals* in generating and utilizing scientific data to improve the health and well-being of the community and broaden their scientific knowledge within the *healthcare system and community setting*. Providing this service requires the application of integrated knowledge in research methods, measurements of health and disease, biostatistics, epidemiology, clinical audit, evidence-based practice, and research ethics. To demonstrate competence in this domain, candidates must possess applied knowledge in planning, problem identification, data collection, analysis, interpretation, report writing, and dissemination of research findings.

Key role/ domain 3: Professionalism (5.5%)

Description: This domain encompasses the professional commitment of *Midwifery professionals* to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of *practice*. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medicolegal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, continuous professional development, and professional values such as integrity, honesty, altruism, humility, and motivation, compassionate, respectful care.

Key role/ domain 4: Leadership and management (5.0%)

Description: This domain encompasses the professional role of *Midwifery professionals* in envisioning a high-quality healthcare system through self-awareness, active participation in healthcare team development and leadership, and managing health systems. Providing this service requires the application of integrated knowledge in continuous quality improvement, effective health system leadership, management, and healthcare ethics. To demonstrate competence in this domain, the candidate shall possess applied knowledge to plan, organize, staff, lead, execute, monitor, and control healthcare resources and activities.

Key role/ domain 5: Health promotion and disease prevention (13.0%)

Description: This domain encompasses the professional role of *Midwifery professionals* in enhancing the health and well-being of patients, communities, and the larger population they serve through health advocacy, disease prevention, health promotion, and the promotion of health equity. Providing this service takes an integrated understanding of determinants of health, health informatics, epidemiology, communicable disease control, and health education.

Table 1: Exam Content for Midwifery Profession

Domain: Patient care		
Sub-Domain: Preconception		
Content	Processes	Tasks
Promote healthy pregnancy and disease prevention	Assessment	Assess a client during preconception care
	Diagnosis	Diagnosing Nutritional Problems and Common Medical Problems
	Management and evaluation	Manage a client during preconception care
Sexual health and violence	Assessment	Assess preconception client for Sexual health problems and violence
	diagnosis	Diagnose preconception client for Sexual health problems and violence
	management and evaluation	Manage and evaluate sexual health problems and violence
Mental health and psychoactive substance use	Assessment, Management, and Evaluation	Assess and manage clients for mental health and psychoactive
Sub-Domain: Family Planning		
Content	Processes	Tasks
Natural Methods	Assessment	Counsel natural FP methods
Modern FP Methods	Assessment	Assess clients for modern FP methods: barrier methods, short-acting FP, long-acting FP, permanent methods, and emergency contraceptives
	Management	Provide modern FP methods: barrier methods, short-acting FP, long-acting FP, permanent methods, and emergency contraceptives
	Evaluation	Evaluate clients utilizing modern FP methods: barrier methods, short-acting FP, long-acting FP, permanent methods, and emergency contraceptives
Post-partum FP	Assessment	Assess clients for postpartum FP methods
	Management	provide post partum FP methods
Sub-Domain: Antenatal care		
Content	Processes	Tasks
Early detection and treatment of problems and complications during pregnancy	Assessment	assess pregnant woman
		Assess a pregnant woman with a problem
	Diagnosis	Diagnosing an abnormal pregnancy
	Management	Manage abnormal pregnancy
Prevention of disease and health promotion	Evaluation	Evaluate a pregnant woman
		Promote a healthy pregnancy
Sub-Domain: Intrapartum care		
Content	Processes	Tasks
Normal labor	Assessment	Assess the woman in labour
	Diagnosis	Diagnose a woman in labour
	Management	Assist a woman in labour
	Evaluation	Re-evaluate the woman in labour
Abnormal labour	Assessment	Assess a woman with abnormal labour
	Diagnosis	Diagnosing abnormal labour
	Management	Manage abnormal labour
	Evaluation	Re-evaluate abnormal labour
Sub-Domain: Post-partum care		
Content	Processes	Tasks
Normal puerperium	Assessment	Assess normal puerperium
	Management	Promote a healthy postpartum period
	Evaluation	Provide comprehensive follow-up for postpartum mothers
Abnormal	Assessment	Assess abnormal puerperium

puerperium	Diagnosis	Diagnosing postpartum complications
	Management	Provide comprehensive care for postpartum mothers
	Evaluation	Evaluate mothers managed for postpartum complications
Sub-Domain: Neonatal and child care		
Content	Processes	Tasks
Essential newborn care	Assessment	examine newborn
	Management	provide essential newborn care
Advanced Newborn Care	Assessment	Assess common neonatal problems
	Diagnosis	Diagnosing neonatal problems
	Management	Manage neonatal problems
	Evaluation	Evaluate a neonate with a problem
Under five cares	Assessment	Assess child development and common childhood illnesses
	Diagnosis	Classify a sick child and assess the child's growth and development
	Management	Provide care for children under five
	Evaluation	Evaluate a sick child
Sub-Domain: Gynecologic and abortion care		
Content	Processes	Tasks
Early pregnancy bleeding	Assessment	Assess a woman with early pregnancy bleeding
	Diagnosis	diagnose ectopic pregnancy and GTD
	Management and evaluation	Manage and evaluate a woman with early pregnancy bleeding
Menstrual and reproductive endocrine abnormalities	Assessment	Assess a woman with menstrual and reproductive endocrine abnormalities
	Diagnosis	diagnose a woman with menstrual and reproductive endocrine abnormalities
	Management	Manage and evaluate a woman with menstrual and reproductive endocrine abnormalities
Gynecologic tumors	Assessment and Diagnosis	Assess and diagnose a woman with gynecologic tumors
	Management	manage a woman with a common gynecologic tumor
Other gynecologic disorders	Assessment	Assess a woman with STIs and other gynecologic disorders
	Diagnosis	diagnose a woman with STIs and other gynecologic disorders
	Management and evaluation	manage a woman with STIs and other gynecologic disorders
Comprehensive Abortion Care	Assessment	Assess clients with abortion
	Diagnosis	diagnose abortion
	Management	provide comprehensive abortion care
	Evaluation	Evaluate clients after comprehensive abortion care
Uro-genital gynecologic disorders	Assessment and Diagnosis	Assess and diagnose a woman with urinary incontinence and an obstetric fistula
	Management and evaluation	manage a woman with urinary incontinence and an obstetric fistula
Sub-Domain: Fundamental Midwifery skills		
Content	Processes	Tasks
Basic midwifery skills	Management	Perform basic midwifery skills
	Evaluation	Evaluate clients after basic midwifery care procedures
First aid	Assessment	Assess clients requiring first aid
	Management	Provide first aid for clients with common emergencies
Domain: Health promotion and Disease prevention		
Content	Processes	Tasks
Community diagnosis	Planning	perform community diagnosis
	Field work/data collection	investigate outbreaks
Community-based intervention	Action plan	Set an action plan for community intervention
	Intervention	Provide intervention for community problems

Domain: Scholar	
Tasks	
Develop a research proposal	
Apply techniques of data collection	
Analyze and interpret data	
Sub-Domain: Professionalism	
Content	Tasks
Professional ethics and medico-legal practice	Function within the legal and ethical framework
Principles of professionalism	Promote principles of professionalism
Communication and collaboration	Provide client-centered care
	Communicate effectively
Domain: LMG	
Tasks	
Plan for health system management	
Execute Plans in Health System Management	
Monitor and evaluate health services using performance indicators	

Sample Questions

1. A 24-year-old mother comes to a family planning clinic to use contraception and seeking counseling about existing options. On history, she reported that she had been using other family planning methods and faced side effects and complications. In addition, she had a history of sexually transmitted infections and her partner also cooperated for any method preferred for her health.

What is the most appropriate family planning method for this woman?

- | | |
|-----------------------------|------------------------------|
| (A) Condom | (C) Tubal ligation |
| (B) Injectable depo provera | (D) Oral contraceptive pills |

Answer key: The answer is **A**

Explanation: As the patient faced side effects and complications on using other family planning methods, and also had a history of sexually transmitted infections, condom is the most appropriate method. This is because Condom is a short acting barrier family planning method used to prevent sexually transmitted infection and pregnancy, and has no side effects and complication(s). On the other hand, Option (B) Injectable depo provera, and option (D) oral contraceptive pills are hormonal contraceptives that have side effects and complications, and don't prevent sexually transmitted infections. In the same vein, Tubal ligation (Option C) is a long acting family planning method which also doesn't prevent sexually transmitted infections, and is recommended for couples who decide not to have more children and/or when the woman has a health problem.

2. A pregnant woman of 32 weeks gestational age comes to antenatal care clinic with a complaint of a gush of watery vaginal discharge before an hour. She had a history of preterm birth. Speculum examination indicated leakage of fluid from the cervix and cervical length was 1 cm. When nitrazine paper immersed in the collected fluid from posterior fornix, it turns from yellow to blue.

What is the most likely diagnosis in this scenario?

- | | |
|---------------------------|-------------------------------------|
| (A) Incontinency of urine | (C) Sexually transmitted infections |
| (B) Hydrorrhea gravidarum | (D) Premature rupture of membranes |

Answer key: The answer is **D**

Explanation: In this scenario, the pregnant woman was beyond 28 weeks of gestational age having a gush of watery discharge, explored through speculum examination and colour change from yellow to blue indicates premature rupture of membranes. Spontaneous rupture of the membranes any time beyond 28th week of pregnancy but before the onset of labour is called premature rupture of the membranes (PROM). On the other hand, Incontinency of urine (option A), takes place as the gestational age increases during pregnancy especially in the late months/weeks and no evidence of colour change. Hydrorrhea gravidarum (option B) is a state

where periodic watery discharge occurs probably due to excessive decidual glandular secretion. Sexually transmitted infections (STI) (option C) are mainly due to unsafe abortion, unclean delivery, poor menstrual hygiene and unhygienic IUD insertion.

3. A 29-year-old multigravida woman at 38 weeks of gestational age (reliable for date) comes to an antenatal care unit with a complaint of vaginal bleeding. She has explained that the bleeding is bright-red with no associated pain. On physical examination, she is healthy looking and her blood pressure is 120/75 mmHg, respiration rate is 17/min and pulse rate is 78/min. The finding, through abdominal examination, has shown cephalic presentation and 90/min; for fetal heart rate. On ultrasound evaluation: fetal heart beat(FHB) are seen, gestational age 38 weeks, amniotic fluid index(AFI) is 8 cm and placenta totally covered the internal os of the cervix.

What is the most appropriate management for this patient?

- (A) Admit her to ward and prepare for laparotomy
- (B) Admit her to ward and prepare for cesarean delivery
- (C) Admit her to ward and prepare for induction of labor
- (D) Admit her to ward and prepare for augmentation of labor

Answer key: The answer is **B**

Explanation: The woman is in the third-trimester or term pregnancy without symptom of true labor but has painless vaginal bleeding which indicates ante-partum hemorrhage (placenta previa). Since the placenta totally covers the internal os of the cervix, admitting the woman and preparing here for emergency cesarean delivery is the most appropriate management in this case. On the other hand, admitting here to ward and preparing for laparotomy (option A) is recommended for patients with uterine atony after delivery with ligation of pelvic vessels. Since internal os of the cervix is totally covered by the placenta, induction and /or augmentation of labour (options C & D) are contraindicated.

4. A 37-year-old multi gravida woman with a gestational age of 37 weeks comes to a health center with a complaint of pushing down pain of seven hours duration. Vaginal examination revealed a cervix of 5 cm dilated with station -2, membrane intact and pulsation of the cord was appreciated.

What is the most likely type of cord accident faced by the woman?

- (A) Occult cord prolapse
- (B) Overt cord prolapse
- (C) Cord presentation
- (D) Cord entanglement

Answer key: The answer is **C**

Explanation: This is because the membrane is intact and pulsation of the cord appreciated below the presenting part of the fetus. On the other hand, occult cord prolapse (option A) occurs when the membranes rupture and the cord descends below the presenting part. Overt cord prolapse (option B) occurs when the prolapsed cord may be palpated in the cervical canal, in the vagina or may be visible at the vulva. Cord entanglement (option D) is excessive cord length without describing the status of the membrane.

5. A Gravida-II woman comes to a health center with a complaint of pushing down pain of seven hours duration at 37 weeks of gestation. At admission, the cervical dilatation was 7cm and she followed with parthograph. After three hours, the cervix became fully dilated. The midwife notified the staffs and transferred the woman to delivery room.

What is the most appropriate next step in the preparation for assisting birth in this case?

- A. Clean the vulva and perineum with antiseptics
- B. Ensure equipment for delivery are available
- C. Sterile draping exposing immediate area around vulva
- D. Wash hands and wear personal protection equipment

Answer key: The answer is **B**

Explanation: In this woman, the fully dilated cervix indicates the woman is on second stage of labour. After notifying the staffs that delivery is imminent and transferring the woman to the delivery room, ensuring availability of equipment for delivery is the next appropriate step in the preparation of the woman for assisted delivery. Clean the vulva and perineum with antiseptics (option A) is among preparation steps but it has to be done after preparing equipment and putting personal protective materials. Sterile draping exposing immediate area around vulva (option C) comes after wearing personal protective and cleaning the vulva. Wash hands and wear personal protection equipment (option D) is essential step in the preparation but it has to follow ensuring availability of equipment for delivery.

6. A 37-year-old Gravida-II Para-I woman at 37 weeks of gestation comes to labour and delivery ward with complaints of severe headache and blurring of vision. On examination, 36 weeks sized gravid uterus, longitudinal lie, and cephalic presentation with no uterine contraction is observed. Vaginal examination revealed closed cervix.

What is the most appropriate step in the management of this woman?

- (A) Follow with parthograph
- (B) Counsel and send her home
- (C) Admit to labour and delivery ward
- (D) Allow to ambulate around the corridors

Answer key: The answer is **C**

Explanation: In this scenario, the woman's complaint of severe headache and blurring of vision at gestational age of 37 weeks indicates the woman has pre-eclampsia with severity feature at term. The most appropriate management for this case is admit to labour and delivery ward (option C). Delivery is recommended in the management of pre-eclampsia with severity feature of GA $\geq$ 37 weeks. Follow up with parthograph (option A) is indicated during active first stage of labor, but in this case, the woman is not in labor; she has no uterine contraction. Counsel and send her home (option B) and allow to ambulate around the corridors (option D) are not appropriate management as the woman has risk of convulsion. Headaches or visual disturbances can precede eclampsia (convulsion).

7. A 34-year-old Para-VI woman comes with a complaint of vaginal bleeding for one hour after giving birth at the local health center. The third stage was managed actively. On physical examination, pulse rate is 120/min and blood pressure is 80/50mmHg with excessive vaginal bleeding observed on physical examination.

What is the most likely diagnosis of the above case?

- | | |
|------------------|----------------------------|
| (A) Lochia rubra | (C) Secondary PPH |
| (B) Primary PPH | (D) Third stage hemorrhage |

Answer key: The answer is **B**

Explanation: In this case, the woman's diagnosis is primary PPH, because she is presented with vaginal bleeding after one hour of delivery with raised pulse rate and falling blood pressure. Primary PPH is hemorrhage which occurs within 24 hours following the birth of the baby which adversely affects the general condition of the patient evidenced by rise in pulse rate and falling blood pressure. Lochia rubra (option A) is a sloughed superficial layer of decidua 2-3 days after delivery. Secondary PPH (option C) occurs beyond 24 hours and within puerperium. Third stage hemorrhage (option D) is bleeding that occurs before expulsion of placenta.

8. A 24-year-old primiparous woman has been on her ninth postoperative day after cesarean delivery for an indication of obstructed labor. She has a complaint of involuntary leakage of urine. Otherwise, she has no dysuria, frequency or urgency of urination. A Sims' speculum shows continuous urine leakage through vagina.

What is the most likely diagnosis in this case?

- | | |
|-----------------------|---------------------------------|
| (A) Obstetric Fistula | (C) Stress urinary incontinence |
| (B) Voiding Disorder | (D) Painful Bladder Syndrome |

Answer key: The answer is **A**

Explanation: In this woman with a history of obstructed labor, the most likely diagnosis is obstetric fistula. Obstetric fistula results from prolonged compression effect on the bladder base between the head and symphysis pubis in obstructed labor. It causes continuous urine escape

through vagina. Voiding Disorder (option B) is difficulty of emptying the bladder which leads to retention of urine, not urinary incontinence. Stress incontinence (option C) is an involuntary escape of urine from the external urinary meatus due to sudden rise in intra-abdominal pressure, but in this case, the woman has urine leakage through vagina. Painful Bladder Syndrome (option D) is chronic inflammatory condition resulting in painful voiding and it may cause urinary incontinence, but in this case, the woman has no painful voiding and the urine leakage is through vagina.

9. Couples come to gynecology clinic with a complaint of being unable to conceive after a year of regular sexual intercourse. The husband has two children from his previous wife. The wife's reproductive history reveals 45-60 days interval of menses which lasts for Eight days without molar symptoms. She has never been diagnosed with STI or cervical neoplasm. Physical examination and Hysterosalpingogram findings are normal. After evaluation, the couples were diagnosed as infertile.

What is the most likely cause of infertility in this couple?

- | | |
|---------------------|--------------------|
| (A) Uterine factor | (C) Ovarian factor |
| (B) Cervical factor | (D) Tubal factor |

Answer key: The answer is C

Explanation: The most likely cause of infertility in this case is ovarian factor. Ovarian factor, ovulatory dysfunctions, includes anovulation or oligo-ovulation. A woman with cyclic menses at an interval of 25 to 35 days and duration of bleeding of 3 to 7 days is most likely ovulating. In this case, the wife has irregular menses which indicates anovulatory cycle. Uterine factor (option A) such as myoma, intrauterine adhesion and congenital anomalies are not identified through examination. Cervical factor (option B) commonly associated with abnormalities in mucus production after treatment for cervical neoplasia, is not observed in this case. Normal finding of Hysterosalpingogram excludes Tubal factor (option D) associated with tubal obstruction and pelvic adhesions.

10. A woman who has no history of antenatal follow up gave birth in a health center. During the newborn assessment, the midwife identified saclike cystic structure covered by a thin-layered membrane which contains spinal cord and meninges in the lumbosacral region. The newborn has weak extremities with diminished tone.

What is the most likely diagnosis in this case?

- | | |
|-------------------|--------------------------|
| (A) Meningocele | (C) Myelomeningocele |
| (B) Encephalocele | (D) Spina bifida occulta |

Answer key: The answer is C

Explanation: In this newborn with protrusion of spinal cord and meninges, the most likely diagnosis is myelomeningocele. Myelomeningocele is a neural tube defect characterized by

protrusion of spinal cord and meninges through a defect in the spinal cord. The newborn may have flaccid paralysis of the lower extremities. Meningocele (option A) is the herniation of the meninges through a defect in the posterior vertebral column. Encephalocele (option B) is protrusion of meninges with or without brain tissue through a midline defect in the skull. Spina bifida occulta (option D) is midline defect of the vertebral bodies without protrusion of the spinal cord or meninges.

11. A woman comes to a health center with complaints of her five-year-old child's diarrhea problem for the last three hours. She reported to a midwife on duty that the diarrhea was abrupt, watery, and looks like rice water.

What would be the most appropriate clinical diagnosis in this case?

- | | |
|----------------|-----------------|
| (A) Cholera | (C) Shigellosis |
| (B) Giardiasis | (D) Amoebiasis |

Answer key: The answer is A

Explanation: Cholera (option A) is an acute illness caused by an enterotoxin elaborated by vibrio cholerae with a clinical manifestation of an abrupt painless watery diarrhea which looks like rice water. Giardiasis (option B) is a protozoan infection principally of the upper small intestine associated with symptoms of chronic diarrhea, steatorrhea abdominal cramps, bloating, frequent loose and pale greasy stools, and fatigue and weight loss. Shigellosis (option C) is an acute bacterial disease involving the large and distal small intestine, caused by the bacteria of the genus shigella comprised of four species or serotypes. Amoebiasis (option D) is an infection due to a protozoan parasite that causes intestinal or extra-intestinal disease and with dysentery; feces are generally watery, containing mucus and blood.

12. A midwife has been assigned to coordinate maternal mortality reduction project in rural Ethiopia with low community mobility. The project is designed to reduce maternal mortality through improvement of contraceptive prevalence and utilization of contraceptives.

What is the most appropriate family planning delivery method in this case?

- | | |
|--------------------|---------------------|
| (A) Work place | (C) Market based |
| (B) Facility based | (D) Community based |

Answer key: The answer is D

Explanation: The objective of the project is to reduce high maternal mortality in the rural area by mobilizing the community and providing short-acting contraceptives. Work place (option A), this is designed to work in the specified place by integrating with a routine activity. This option doesn't go with the objective mentioned in the stem and it can't be the answer. Facility based

(option B) is one of the approved outlets for providing family planning services where it is stipulated that “all health institutions in Ethiopia can provide contraceptives based on their capacity. Here we are dealing with a separate project and can’t fit the project and is not the answer. Market based (option C) is designed to fill the gap between the public sector and for profit/private sector and is not the answer.

13. A 26-year-old Gravida II and Para I woman comes to a gynecology ward due to a complaint of absence of fetal movement. Ultrasound examination revealed negative fetal heartbeat. Thus, the woman was told to choose between an immediate induction of labour or to come back when labor starts spontaneously.

What ethical principle is implemented to manage the situation in this scenario?

- | | |
|-----------------|--------------------------|
| (A) Justice | (C) Non-Maleficence |
| (B) Beneficence | (D) Respect for autonomy |

Answer key: The answer is **D**

Explanation: Respect for autonomy is one ethical principle where clients exercise their own self-determination. In the scenario, the client was given a choice where she chooses between treatment options without coercion, and exercising her self-determination by deciding what needs to be done for her own health. That means, the professional showed respect for her autonomy, and she exercised her right for self-determination. On the other hand, Justice (Choice A) is an act of treating all patients equally, was not discussed in the scenario. In the same manner, Beneficence (choice B), and non-maleficence (Choice C) might be considered but little is said in the scenario.

14. A midwife working as a head of health center strives to improve clients’ satisfaction on the care provided during pregnancy. He collects sufficient information from the clients for future improvement and he asks partners for technical and financial support. Hence, the midwife decides to improve client’s waiting time, counseling services and provide services as per the standard in the coming year.

What is the most appropriate management function the midwife implemented in this case?

- | | | | |
|----------------|--------------|-------------|--------------|
| (A) Organizing | (B) Planning | (C) Leading | (D) Staffing |
|----------------|--------------|-------------|--------------|

Answer key: The answer is **B**

Explanation: Before a manager tackle any of the other functions, he or she must first devise a plan that specifies the necessary resource allocation, schedule, tasks, and other actions. In this scenario, a midwife performed necessary ground works to increase clients’ satisfaction by improving client’s waiting time, counseling services and providing services as per the standard in the coming year. Organizing (option A), here the midwife needs to secure technical and financial

support from the partners. Hence, the main issue does not deal with the task of organizing; for this reason option „A“ may not be the answer. On the other hand, there is nothing mentioned in the scenario about motivating and guiding the employees. Thus, leading (option C) is also not the answer. Staffing (option D), deals with assigning human resource to respective tasks which is not mentioned in the scenario. So, it is not the answer.

15. A study has been conducted to assess the difference on exposure status to cigarette smoking between lung cancer patients and healthy individuals. After selecting the representative sample from those with and without lung cancer, the investigator collects important information about their exposure status.

What is the most appropriate study design for this study?

- | | |
|------------------|---------------------|
| (A) Cohort | (C) Experimental |
| (B) Case control | (D) Cross sectional |

Answer key: The answer is **B**

Explanation: The case control study design is an observational study to identify the risk factor of a disease. The study includes people with a disease of interest and a suitable control group of people unaffected by the disease. The investigators collect data on disease occurrence at one point in time and exposures at a previous point in time. In the above scenario the investigator collects data on the previous exposure status of smoking among lung cancer patients and healthy individuals which makes the design to be case control study. But in Cohort (option A), the study starts from the exposure to identify the risk factor of disease. Similarly, for Experimental (option C), the study needs certain type of intervention by the researcher to identify the risk factor of disease whereas Cross sectional (Option D) is a design for a prevalence study. Therefore, the answer for the question is option B, case control.

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